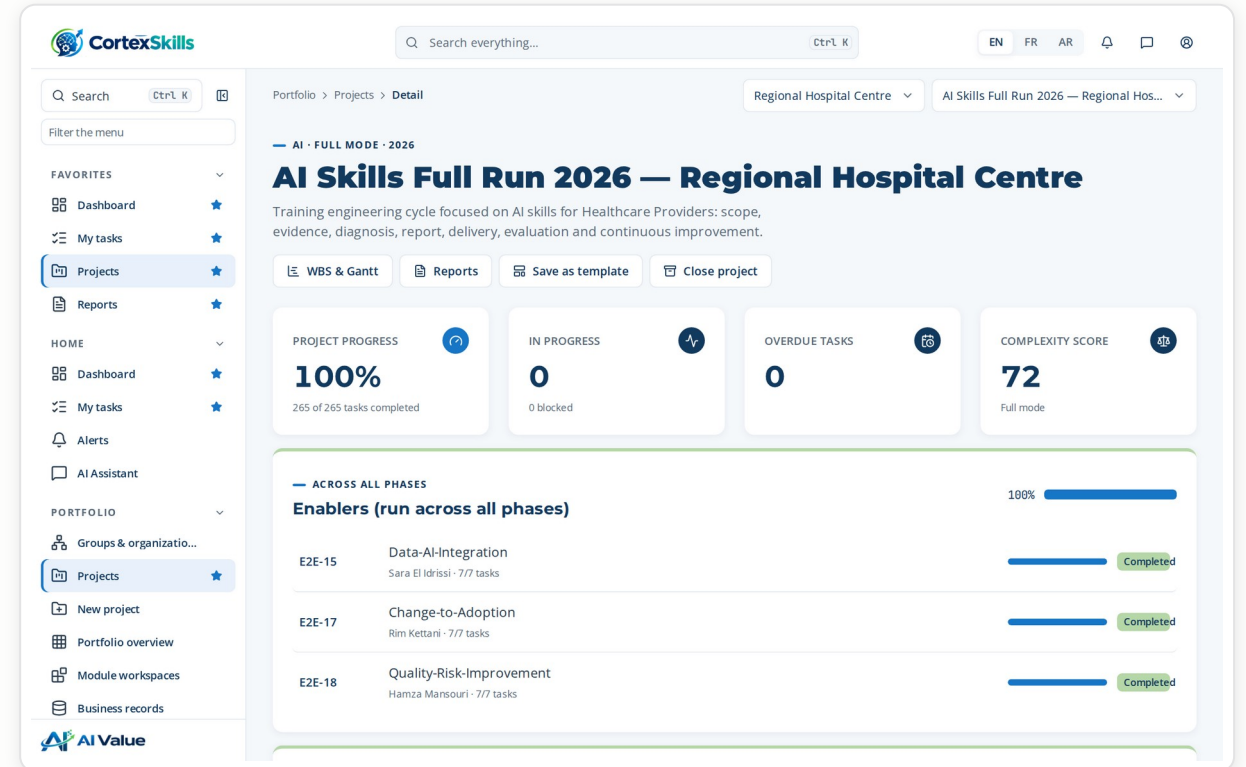


AI skills training engineering at a regional hospital

Regional Hospital Centre: from scope to evaluation — the project, the Training Engineering Report, the Training Plan and every document produced with CortexSkills



How the case is presented



01

The hospital and the project

Context, charter, scope, method, schedule



02

Diagnosis

Questionnaires, maturity, skill gaps, needs



03

Training Engineering Report

Structure, roadmap, risks



04

Training Plan

Programs, trainings, agendas, budget



05

Delivery and evaluation

Kirkpatrick, certifications, lessons



06

Documents produced

37 published documents

A complete AI skills cycle, documented end to end

Regional Hospital Centre · 3,400 employees · AI Skills Full Run 2026 — Regional Hospital Centre



of the run completed — 35 end-to-end processes



people answered the questionnaires (93 %)



trainings in 2 programs, prioritized by impact



training budget; 1,192,662 spent

What the case shows



Diagnosis: “Information systems” is the weakest axis (2 / 5); the largest skill gap is “AI bed and staff capacity planning” (average gap 3.0).



10 training needs were validated and grouped into 6 themes, delivered in 4 roadmap initiatives over three waves.



Kirkpatrick results: 86 % · 93 % · 84 % for reaction, learning and behaviour.



37 documents published, 1,060 pages in total; the Training Engineering Report alone has 198 pages.

01

The hospital and the project

Context, charter, scope, method and schedule

Regional Hospital Centre

Regional Hospital Centre operates in the Healthcare Providers sector, in Région, with 3400 employees.

Topic	Content
Vision	Become the reference of Healthcare Providers in the region, with teams trained to the best standards.
Mission	Deliver safe, compliant service through skilled people and controlled processes.
Values	Commitment · Excellence · Sharing
Context	Strong competition, stricter ISO 7101 audits and the arrival of AI triage and clinical decision support.
Products — existing	Core services of Clinical Care & Nursing
Products — new	Digital services supported by AI triage and clinical decision support
Market segments	Large accounts · Public sector · Individual customers

Organization and headcount

Function	People
Operations	3
Quality & HSE	3
Finance	2
Human Resources	2
Sales & Marketing	2
Information Systems	2
Clinical Care & Nursing	2

AI Skills Full Run 2026 — Regional Hospital Centre

Training engineering cycle focused on AI skills for Healthcare Providers: scope, evidence, diagnosis, report, delivery, evaluation and continuous improvement.



72

complexity score (out of 100): full run with gates



40

stakeholders registered and consulted



17

scope criteria approved in the Scope of Work



MS · MO · OP

decision levels covered (strategic, middle, operational)

Strategic objectives



SO-1 — Adopt AI triage and clinical decision support across Clinical Care & Nursing by end 2027 (horizon 2027-12-31)



SO-2 — Zero major findings in ISO 7101 audits (horizon 2026-12-31)

Six phases, each closed by a gate with its deliverables

The training engineering mission follows the phases below, each closed by a gate with its deliverables.

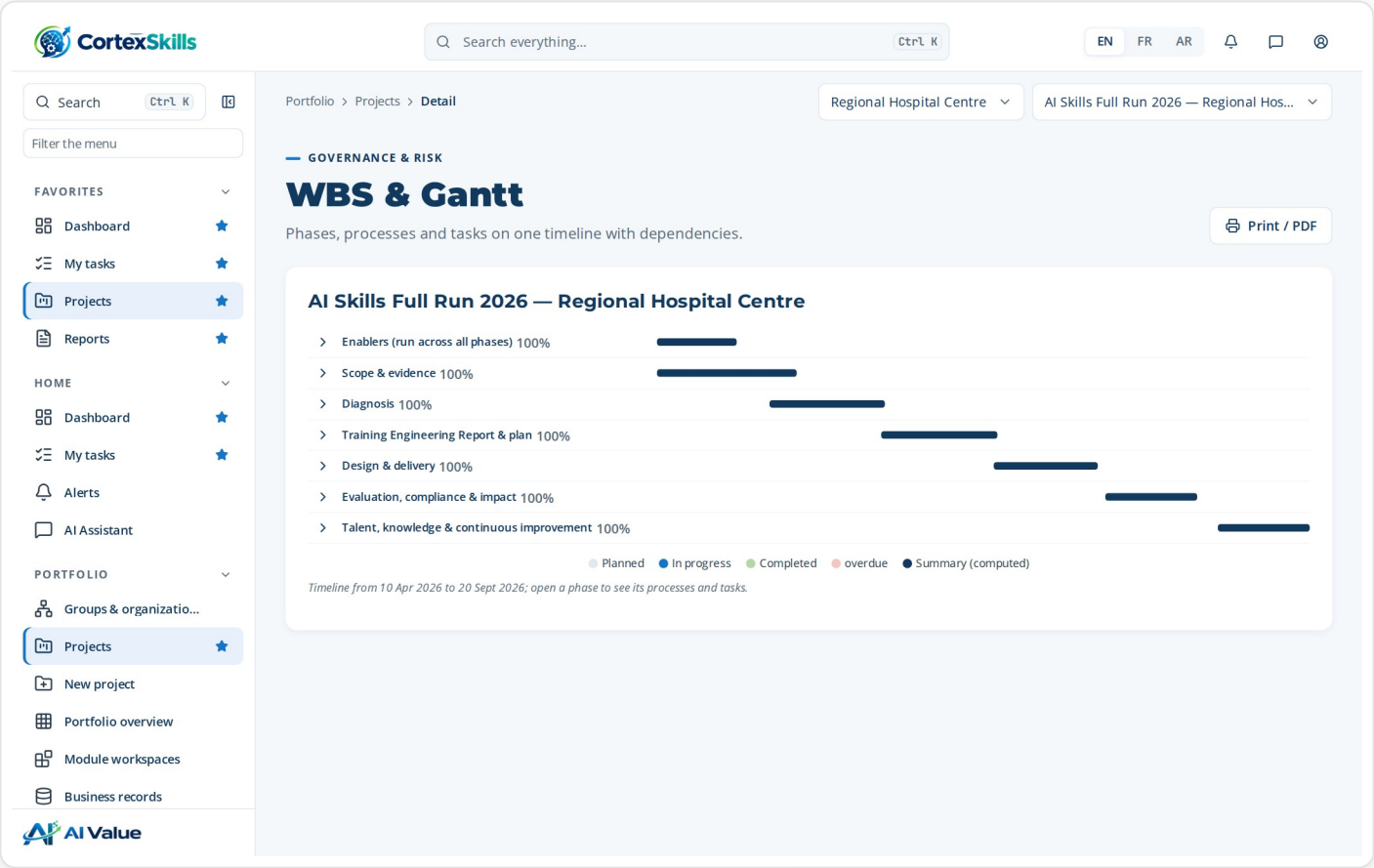
Phase	End-to-end processes	Progress
1. Scope & evidence	E2E-32 · E2E-33 · E2E-19 · E2E-24	100%
2. Diagnosis	E2E-20 · E2E-25 · E2E-26 · E2E-27 · E2E-28 · E2E-29 · E2E-02 · E2E-21	100%
3. Training Engineering Report & plan	E2E-01 · E2E-03 · E2E-30 · E2E-22 · E2E-23 · E2E-16 · HCPR-E2E-02	100%
4. Design & delivery	E2E-04 · E2E-05 · E2E-06 · E2E-13 · E2E-14	100%
5. Evaluation, compliance & impact	E2E-07 · E2E-08 · E2E-09 · HCPR-E2E-01	100%
6. Talent, knowledge & continuous improvement	E2E-10 · E2E-11 · E2E-12 · E2E-31	100%

Source: Training Engineering Report, section 1.5 “The current mission”.

Work packages and the Gantt chart

Work package	Start	End	Progress
Scope & evidence	2026-04-10	2026-05-07	100 %
Diagnosis	2026-05-08	2026-06-04	100 %
Report & plan	2026-06-05	2026-07-02	100 %

Sessions	Date
S-01 AI triage and clinical decision ...	2026-07-19
S-02 AI medical imaging assistance	2026-07-26
S-03 AI triage and clinical decision ...	2026-08-02
S-04 AI medical imaging assistance	2026-08-09



WBS and Gantt of the project in CortexSkills.

02

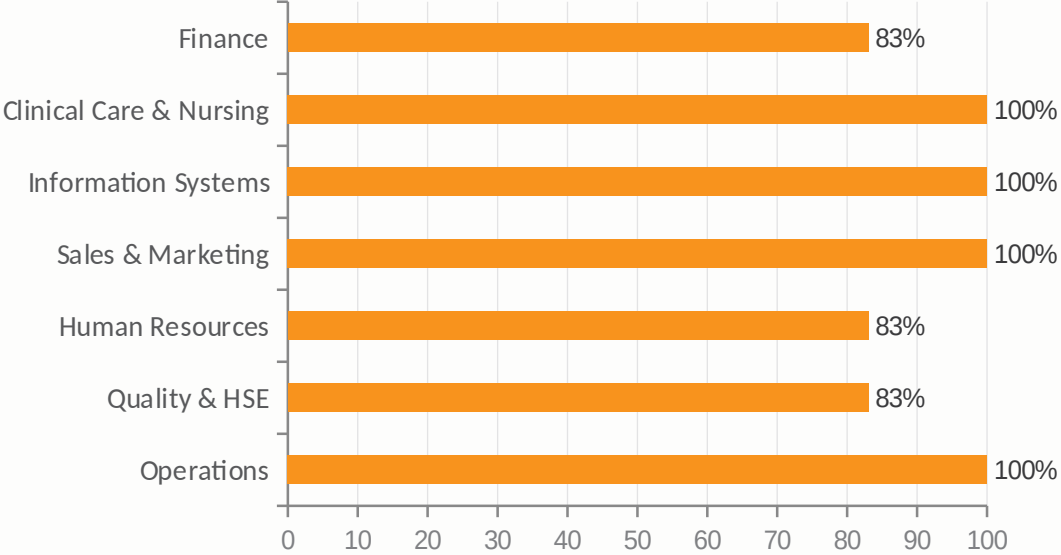
Diagnosis

Who answered, where the hospital stands, which skills are missing

37 of 40 people answered

40 people were invited to answer; 37 answered.

Function	General Manager	Management	Team member	Responded
Operations	1	3	2	6
Quality & HSE	0	4	2	5
Human Resources	0	4	2	5
Sales & Marketing	0	4	2	6
Information Systems	0	3	2	5
Clinical Care & Nursing	0	4	1	5
Finance	0	4	2	5

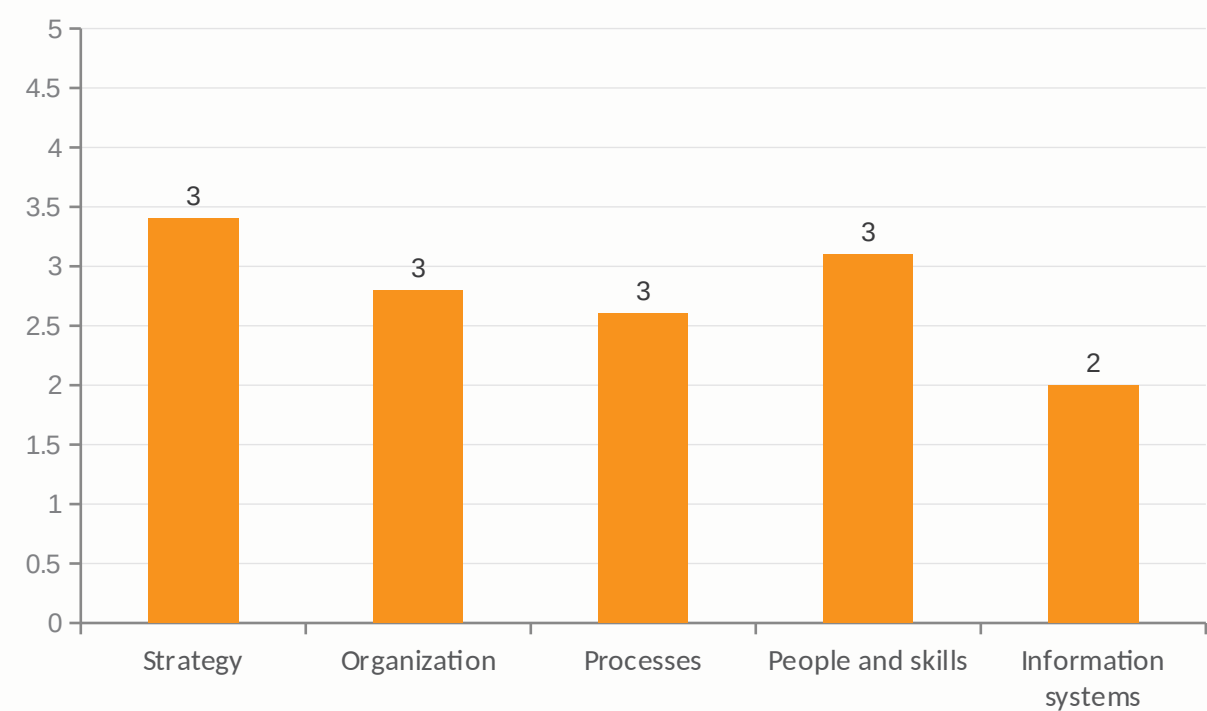


Response rate by function.

Channel	Invited	Responded	Rate	Completeness
Face-to-face	12	12	100 %	95 %
Application	23	23	100 %	91 %
Email	2	2	100 %	77 %

Source: TER sections 1.6, 1.12 and 1.13.

Five axes scored, maturity one level below target



Diagnostic score by axis (TER 1.9).

Axis	Score	Reading
Strategy	3.4	Strength to build on
Organization	2.8	Partly in place
Processes	2.6	Partly in place
People and skills	3.1	Partly in place
Information systems	2	Priority for development

Model	Current	Target	Gap
CMM maturity (CMM)	3	4	1
Leadership maturity (Leadership)	3	4	1

Maturity models (TER 1.10).

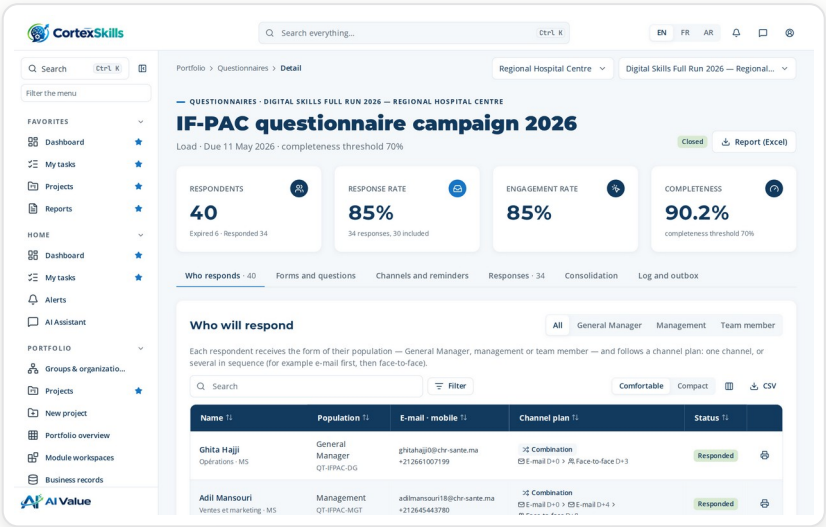
From measured gaps to a validated needs register

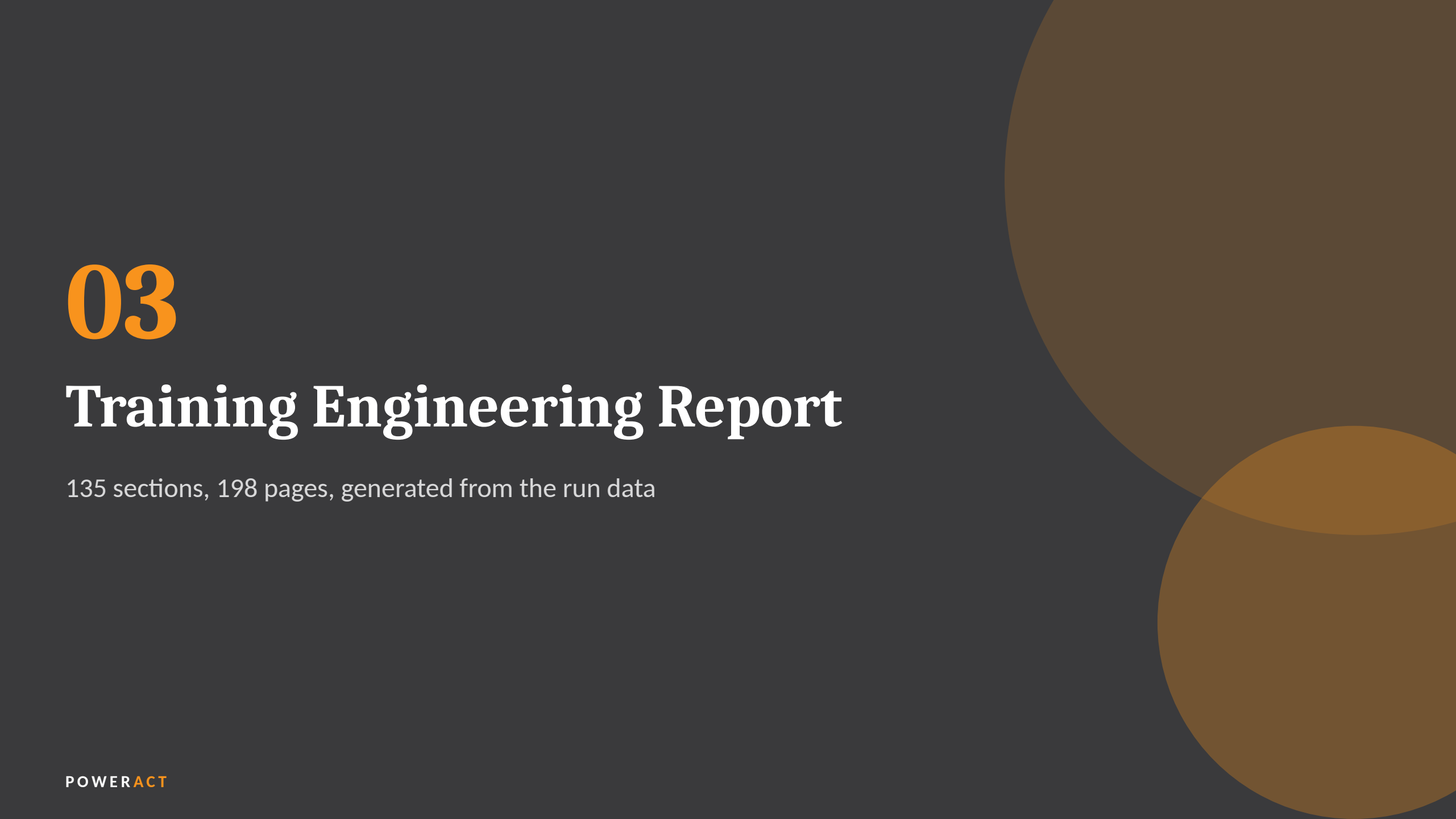
Competency	People	Avg gap	Critical
AI medical imaging assistance	3	2.0	0
AI bed and staff capacity planning	2	3.0	2
AI triage and clinical decision support	3	1.0	0

Skill gaps by competency (TER 3.40).

#	Theme	Source	Impact	Status
1	AI bed and staff capacity planning	Diagnostic	5	Validated
2	Responsible AI and data protection	Skills gap	5	Validated
3	AI bed and staff capacity planning	GPEC	5	Validated
4	AI medical imaging assistance	Demand	4	Validated
5	Prompting and generative AI at work	SWOT	4	Validated
6	AI medical imaging assistance	Diagnostic	4	Validated
7	AI triage and clinical decision support	Skills gap	3	Validated
8	AI literacy for all staff	GPEC	3	Validated
9	AI triage and clinical decision support	Demand	3	Validated
10	AI literacy for all staff	SWOT	3	Validated

Training needs register (TER 3.41): every need traced to its source.





03

Training Engineering Report

135 sections, 198 pages, generated from the run data

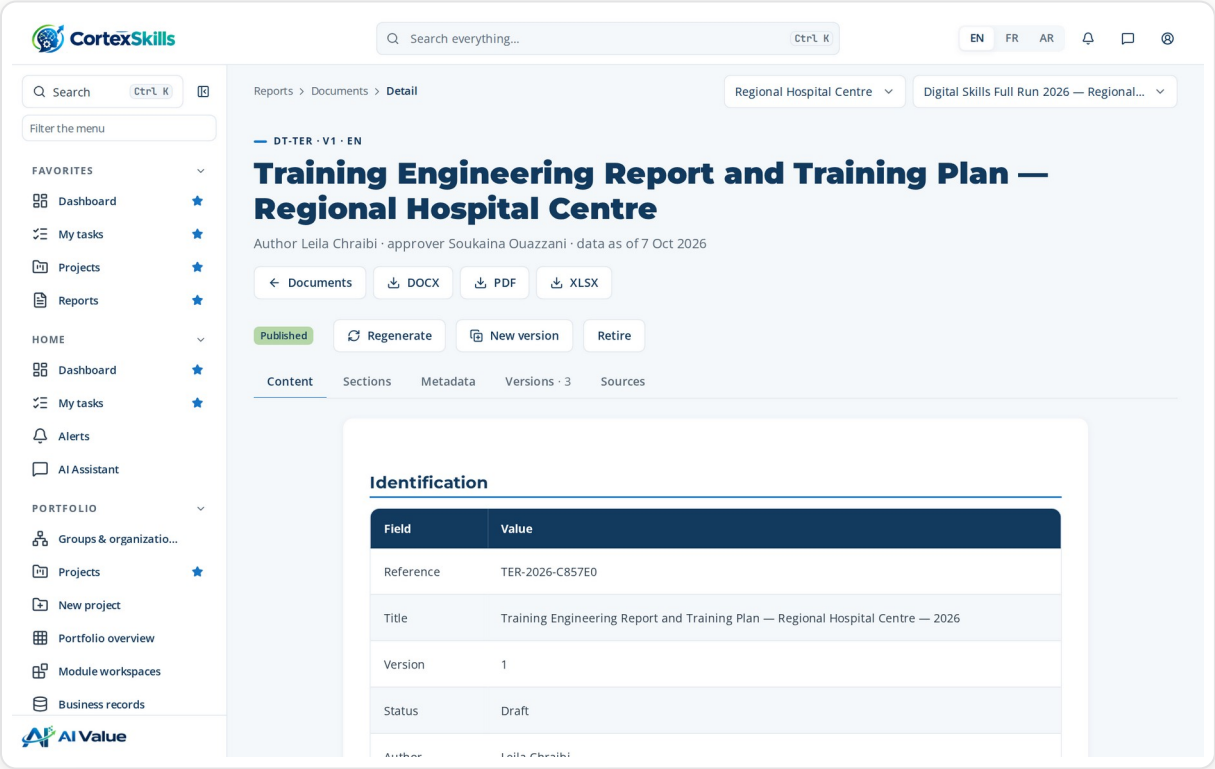
STRUCTURE OF THE REPORT

Training Engineering Report and Training Plan — Regional Hospital Centre

Version 1 · author Leila Chraibi · approved by Soukaina Ouazzani

Chapter	Sections
1. Description of the company	14
2. The previous training plan	2
3. Competence needs	50
4. Perspectives	3
5. The prioritized training plan	23
6. Risks, controls and indicators	2
7. Annexes — interview forms	38

Plus identification, data sources and approval.



The report in the document editor.

Four initiatives deployed in three waves

The perspectives take into account the strategic orientations, the SWOT and improvement axes, the HR orientations and the recommendations made in the face-to-face meetin...

Wave 1

- AI triage and clinical decision support — impact 5
- AI literacy for all staff — impact 5

Wave 2

- AI medical imaging assistance — impact 4

Wave 3

- AI bed and staff capacity planning — impact 3

#	Improvement axis	Implemented	Wave
1	AI triage and clinical decision support	60%	1
2	AI medical imaging assistance	65%	2
3	AI bed and staff capacity planning	70%	3
4	AI literacy for all staff	75%	1

Source: TER chapter 4 and section 4.1.

Each risk reduced by named controls

ID	Risk or opportunity	Category	Inherent	Residual	Controls
RISK-014	Low learner and user adoption	Adoption	16	8	CTRL-081
RISK-017	Critical knowledge lost with expert departure	Operational	16	8	CTRL-070; CTRL-082
RISK-001	Unauthorized access to tenant data	Security	20	6	CTRL-001; CTRL-002; CTRL-048; CTRL-011
RISK-005	Training plan misaligned with strategy	Strategic	16	6	CTRL-049; CTRL-057; CTRL-076
RISK-006	Inaccurate skills data leading to wrong needs	Data quality	15	6	CTRL-064; CTRL-045
RISK-030	Position descriptions diverge from reality	Organizational	12	6	CTRL-066
RISK-007	Unverified AI content in the Training Engineering Report	AI	16	5	CTRL-041; CTRL-056
RISK-009	Expired mandatory certification	Compliance	20	5	CTRL-053; CTRL-054
RISK-031	Site deployment deviates from group plan	Operational	12	5	CTRL-071

Top 9 of 35 risks by residual score (TER chapter 6); 61 project indicators follow in section 6.1.

04

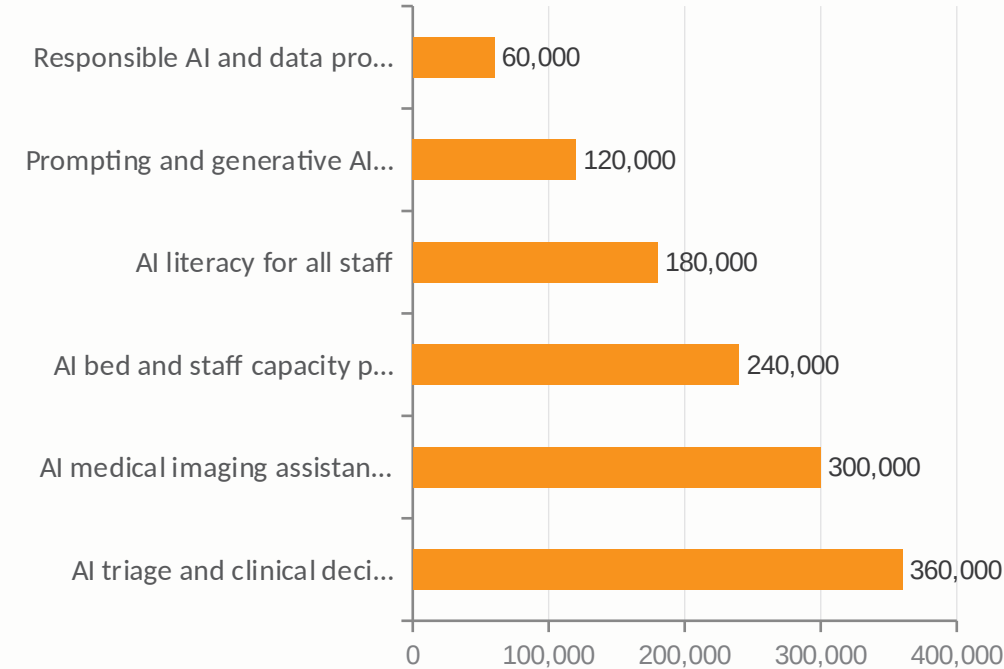
Training Plan

The plan 2026 holds 2 programs and 6 trainings, 12 days per group in total and 44 group-days to deliver, for a budget of 1,260,000.

Six themes ranked by impact and urgency

Plan 2026 — status Locked — budget 1,260,000.

#	Theme	Days	Groups	Budget
1	AI triage and clinical decision support	3	6	360,000
2	AI medical imaging assistance	2	4	300,000
3	AI bed and staff capacity planning	2.5	3	240,000
4	AI literacy for all staff	1.5	3	180,000
5	Prompting and generative AI at work	1	2	120,000
6	Responsible AI and data protection	2	2	60,000



Budget by theme.

Two programs, six trainings, clear prerequisites

Code	Program	Strategic axis	Trainings
PRG-1	AI skills for Clinical Care & Nursing	Execution of the core function	TR-AI-01, TR-AI-02, TR-AI-03
PRG-2	AI foundations for all staff	Leadership, performance and responsible use	TR-AI-04, TR-AI-05, TR-AI-06

ID	Training	Level	Duration	Prerequisites
TR-AI-01	AI triage and clinical decision support	Intermediate	3 day(s)	Basic digital literacy; the Foundation training is recommended
TR-AI-02	AI medical imaging assistance	Intermediate	2 day(s)	Basic digital literacy; the Foundation training is recommended
TR-AI-03	AI bed and staff capacity planning	Advanced	2.5 day(s)	TR-AI-01 or equivalent experience
TR-AI-04	AI literacy for all staff	Foundation	1.5 day(s)	None — open to all staff
TR-AI-05	Prompting and generative AI at work	Foundation	1 day(s)	None — open to all staff
TR-AI-06	Responsible AI and data protection	Intermediate	2 day(s)	Basic digital literacy; the Foundation training is recommended

Source: Training Plan “Programs” and TER section 5.1.

AI triage and clinical decision support — half-day by half-day

22 activities over 3 day(s): lectures, quizzes and workshops on the hospital's own cases.

Day 1 — morning

Lecture · Foundations of AI triage and clinical decision support: key c... (75 min)
Lecture · Why AI triage and clinical decision support matters for Clini... (45 min)
Quiz — key concepts of AI triage and clinical decision support (20 min)
Workshop — map today's practices in Clinical Care & Nursing a... (75 min)

Day 1 — afternoon

Lecture · Methods and tools of AI triage and clinical decision support (75 min)
Lecture · Worked example from the Healthcare Providers sector (40 min)
Quiz — choosing the right method (20 min)
Workshop — apply the method to a real case from Regional Hosp... (80 min)

Day 2 — morning

Lecture · Governance, risks and ISO 7101 requirements (60 min)
Lecture · Data, quality and evidence to keep (45 min)
Quiz — compliance and risk scenarios (20 min)
Workshop — build the control checklist of your process (80 min)

Day 2 — afternoon

Lecture · Deploying AI triage and clinical decision support: roles, cha... (60 min)
Lecture · Indicators that measure the results (45 min)
Quiz — adoption and indicator cases (20 min)
Workshop — design the indicator dashboard of your team (80 min)

Day 3 — morning

Lecture · Advanced practices and lessons learned in Clinical Care & Nur... (75 min)
Quiz — advanced cases (25 min)
Workshop — solve a critical incident scenario (100 min)

Day 3 — afternoon

Lecture · Transfer to the workplace: the 30-60-90-day plan (60 min)
Quiz · Final assessment (Kirkpatrick level 2) (30 min)
Workshop — individual action plan and peer review (90 min)

Why each audience should take the training

Training TR-AI-01 — behaviour, pain points and hopes of each persona



Executive sponsor — General Manager

Decides on facts presented in short steering meetings; delegates execution to the management team; follows a few indicators.

Training spend with no visible link to the strategic objectives; results of the previous plan hard to prove; ISO 7101 audits that expose competence gaps.

A prioritized plan that serves the strategy, a measurable return, and a management team able to lead the change.



Manager — director, head of department, process owner

Balances daily operations and improvement projects; prefers practical, short formats; relies on key people in the team.

Team absent from the job during long trainings; skills gaps on critical activities of Clinical Care & Nursing; processes not documented or without an owner.

Trainings that solve the function's real problems, applied on the job the following week, with indicators the manager can follow.



Team member — Clinical Care & Nursing

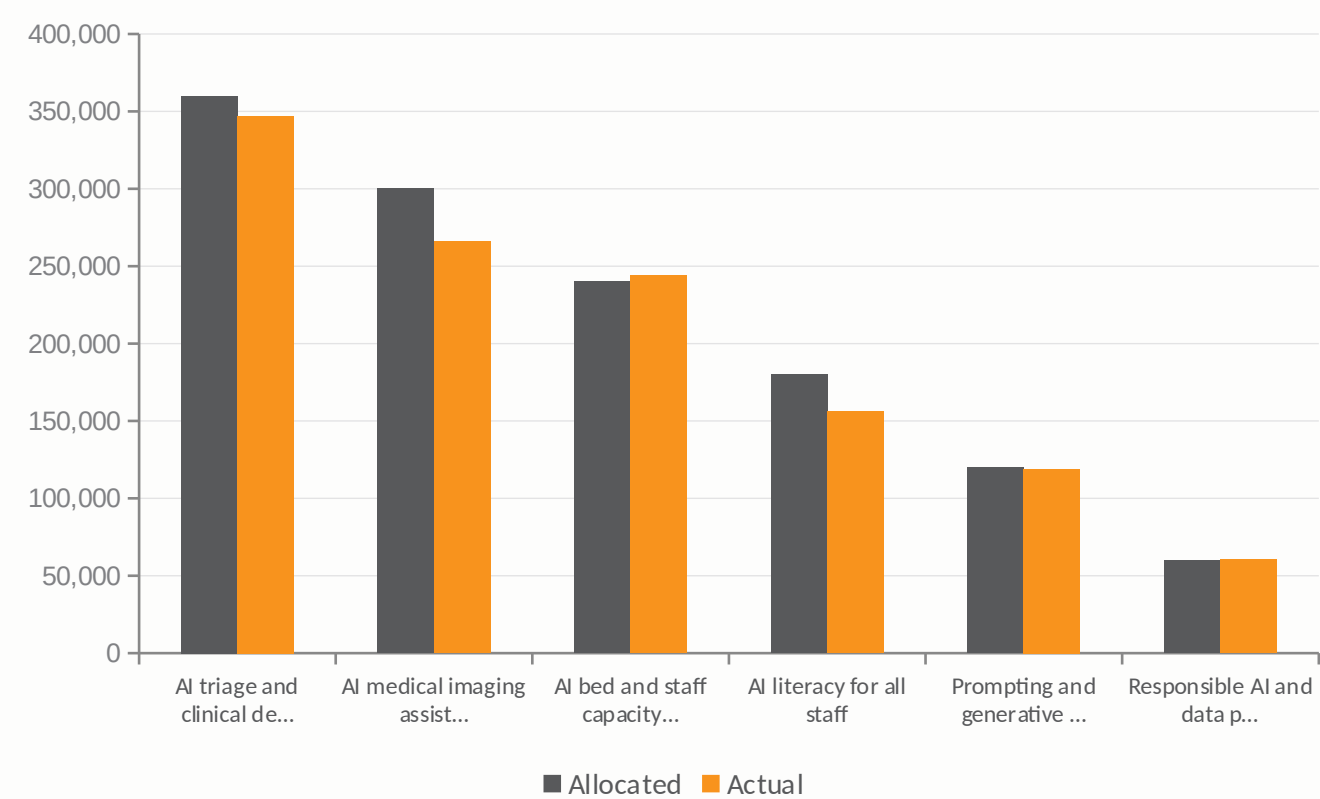
Learns by doing and from peers; uses a phone more than a computer; wants to know what changes in daily work.

New tools introduced without explanation; theoretical trainings far from the tasks; fear of making mistakes with AI triage and clinical decision support.

Practical skills recognized by a certificate, less rework, and a clear path to the next position.

Allocated, committed, spent and refunds expected

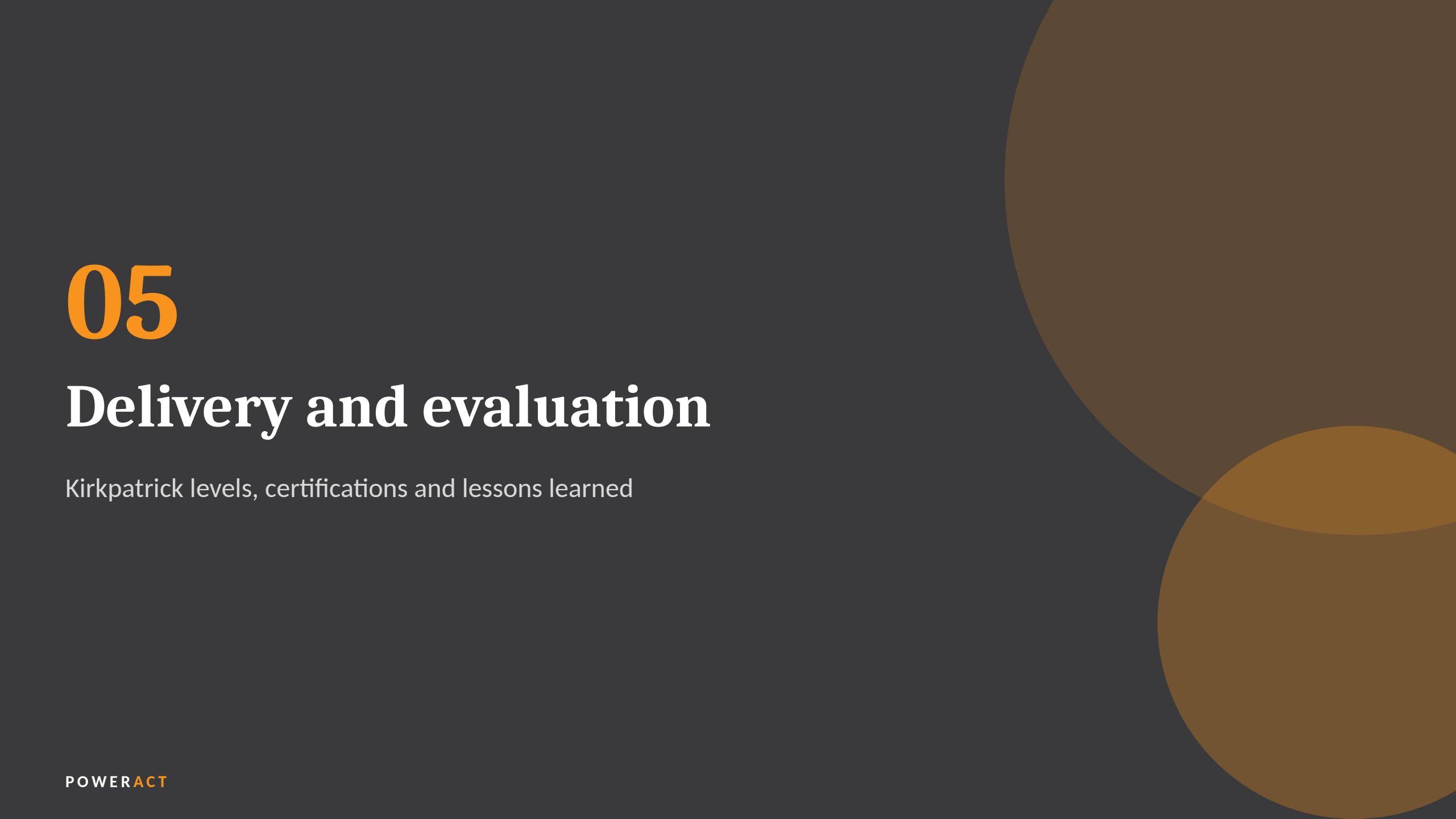
Allocated 1,260,000, committed 1,260,000 (100 %), spent 1,192,662 (95 % of planned); expected refunds 352,800.



Allocated versus actual by training (TER 5.7).

Line	Actual	Refund
AI triage and clinical decisi...	347,127	100,800
AI medical imaging assistance	266,091	84,000
AI bed and staff capacity pla...	243,800	67,200
AI literacy for all staff	156,192	50,400
Prompting and generative AI a...	118,913	33,600
Responsible AI and data prote...	60,539	16,800
Total	1,192,662	352,800

Provider	Accredited until	/5
Atlas Formation Conseil	2027-04-25	4.2
Maghreb Learning Partners	2027-08-03	3.8

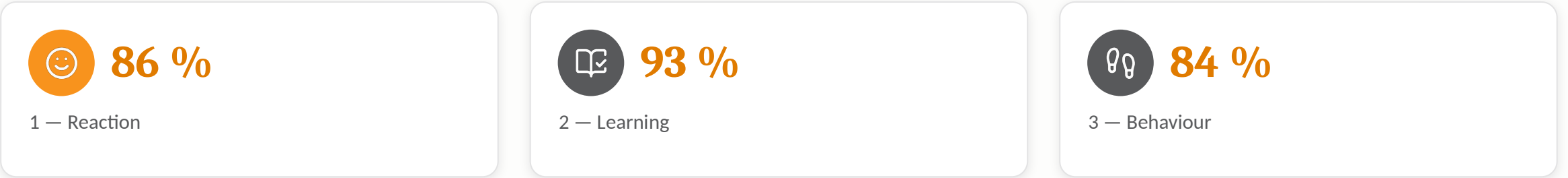


05

Delivery and evaluation

Kirkpatrick levels, certifications and lessons learned

Results measured at three Kirkpatrick levels



Level	What is measured	When	Instrument	Results
1 — Reaction	Satisfaction and perceived usefulness	End of each session	Satisfaction questionnaire	2 × 86 %
2 — Learning	Knowledge and skills acquired	End of the training	Pre- and post-test, case study	2 × 93 %
3 — Behaviour	Use of the skills at the workstation	Three months after	Manager observation grid	2 × 84 %
4 — Results	Effect on the indicators of the function	Six to twelve months after	Indicators of the function (KPI)	—

Certifications (TER 5.10): 4 valid, 1 expiring, 1 expired — the expired one triggers an alert and a renewal session.

REX — AI triage and clinical decision support



What went well

The stakeholder register and multi-channel distribution lifted the response rate.



What did not

SWOT inputs arrived late from one function.



What we will do next time

Name a backup for every function head at kickoff.

06

Documents produced

37 documents published in English, each traced to its data

Every end-to-end process leaves its document

All published, versioned and approved by a second person — 1,060 pages.

Code	Document	Pages
DT-E2E-01	Annual L&D plan and budget	28
DT-E2E-02	Skills gap diagnosis	23
DT-E2E-03	Training needs report and roadmap	28
DT-E2E-04	Curriculum design document	24
DT-E2E-05	Training delivery report	28
DT-E2E-06	Learner engagement report	30
DT-E2E-07	Training evaluation report	23
DT-E2E-08	ROI and business impact report	23
DT-E2E-09	Compliance and certification register	22
DT-E2E-10	Internal mobility and career paths	25

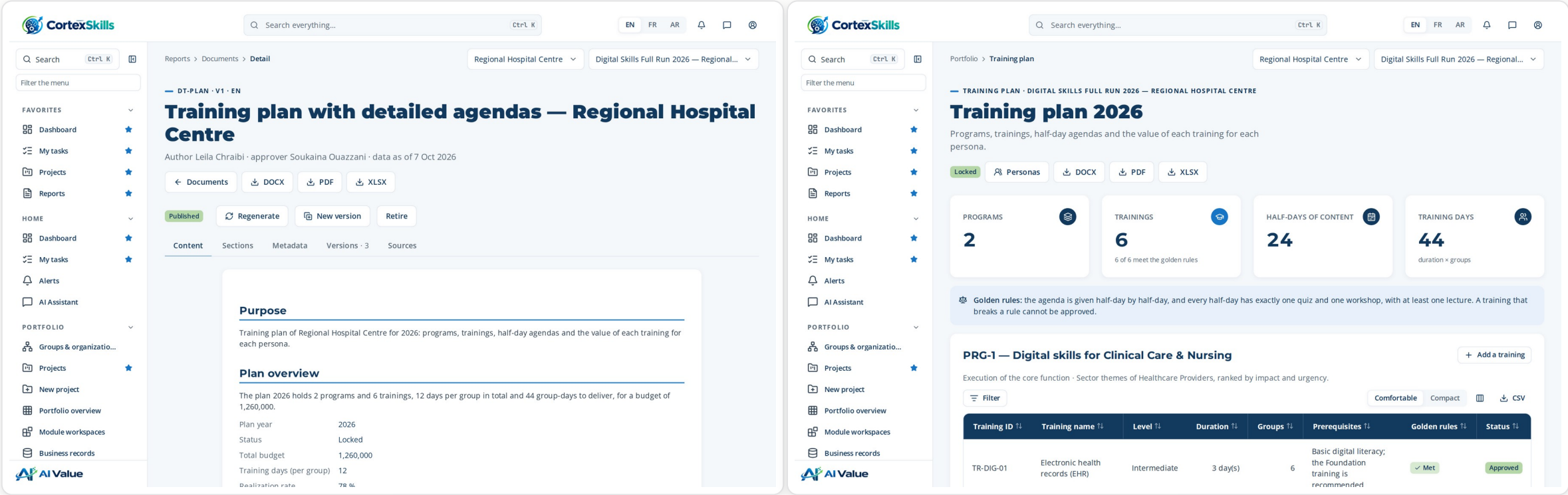
Code	Document	Pages
DT-E2E-11	Onboarding programme record	21
DT-E2E-12	Knowledge capture and transfer record	22
DT-E2E-13	Content catalogue and lifecycle	22
DT-E2E-14	Extended enterprise training report	22
DT-E2E-15	Data, AI and integration governance	24
DT-E2E-16	Vendor management file	20
DT-E2E-17	Change and adoption plan	23
DT-E2E-18	Quality, risk and improvement report	30
DT-E2E-19	Consulting engagement closure report	25

Documents produced (continued)

Code	Document	Pages
DT-E2E-20	Multi-axis diagnostic and maturity report	26
DT-E2E-21	Previous plan impact evaluation	25
DT-E2E-22	Multi-site deployment plan	24
DT-E2E-23	Regulatory submission file	22
DT-E2E-24	Competitive intelligence note	23
DT-E2E-25	Job architecture and competency map	23
DT-E2E-26	Forward planning of jobs and skills (GPEC)	24
DT-E2E-27	Training demand consolidation	21
DT-E2E-28	Performance assessment and RAG dashboard	22
DT-E2E-29	Functional SWOT and improvement axes	23

Code	Document	Pages
DT-E2E-30	Prioritized resource plan	24
DT-E2E-31	Post-engagement roadmap	24
DT-E2E-32	Scope of work and stakeholder register	31
DT-E2E-33	Questionnaire campaign report	29
DT-TER	Training Engineering Report and Training Plan	198
DT-PLAN	Training plan with detailed agendas	34
DT-AUDIT	Audit report	5
DT-MASTER	Master list of documented information	19

The Training Plan as the users see it



Training plan with detailed agendas — Regional Hospital Centre

Training plan screen: programs, trainings and sessions.

What the hospital has at the end of the cycle

-  A Training Engineering Report of 198 pages, built from 37 answers, 40 stakeholders and the diagnosis of every function.
-  A locked Training Plan: 6 trainings, half-day agendas, a value proposition per persona and a budget of 1,260,000.
-  Evaluation at three Kirkpatrick levels, with certifications followed and alerts on expiry.
-  37 published documents ready for an accreditation audit (ISO 7101, JCI), each traced to the data and the step that produced it.
-  Next: waves 2 and 3 of the roadmap and the level-4 (results) evaluation six to twelve months after the trainings.